

Lung Cancer *Monthly*

ISSUE 01 · JANUARY 2026

The month the action moved upstream.

No new approvals. Two filings opened. One trial result that redraws an edge — mapped from public sources, in the open.

- Ivonescimab's US filing: the bispecific read ⁵
- BR.31 publishes: the numbers behind the adjuvant miss ¹⁸
- JPM week: what the money was watching ¹⁵
- The decision calendar: two dates that matter in 2026 ^{4,6}
- Screening, in plain language ²⁴



In this issue

Fourteen sections — from the month's six numbers to every source behind them.

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About this publication

Lung Cancer Monthly is published by VigilLine, an independent healthcare competitive-intelligence practice. Each issue maps one month of public developments in lung cancer for the people who treat it, live with it, and invest in it.

Not medical advice. Informational and professional use only.

Method, briefly. News is taken from primary records — regulators, sponsors, journals — wherever they exist; trade coverage only where primary records do not. Superscripts like 5 index the 38-source ledger on slides 19–20; every one is a live link.

January 2026, in six numbers

One month, compressed: the filings, the miss, the silence — and what it all sets up.

0 NEW LUNG APPROVALS No new FDA approval in lung cancer this month — the wait is the story.⁴	Nov 14 IVONESCIMAB · PDUFA DATE BLA accepted for review (Jan 29) — the decision now on the calendar.⁵	Sep 18 ZIDESAMTINIB · PDUFA DATE NDA accepted (Jan 12) for ROS1-positive NSCLC — a targeted decision date.⁶
0.93 BR.31 · HAZARD RATIO Adjuvant durvalumab missed its endpoint (P = .64) — published Jan 14.¹⁸	2 CONGRESS WEEKS ASCO GI opened the year quietly; JPM week set the capital agenda.^{22,15}	38 PUBLIC SOURCES Every claim in this issue traces to an openable public record.

“The month’s real output was optionality: one more shot on goal in EGFR-mutant disease, one cleaner map of where adjuvant immunotherapy does not help, and two dates on the calendar that will define the year.”

VIGILLINE — MONTH SUMMARY

How to read this deck. Small superscript numbers are source indexes: 5 points to entry [5] on slides 19–20 — click it. Charts, tables and numbers carry the same indexes as the full written issue.

Non-small cell lung cancer: three stages, three different games

The same disease behaves differently at each stage — so the treatment goals differ too.

EARLY STAGE

Surgery first, then the wrap-around question. Adjuvant and neoadjuvant strategies are part of the plan at diagnosis — BR.31 tested one of them, and missed.¹⁸

LOCALLY ADVANCED

Chemoradiation, then consolidation. The PACIFIC-era standard holds the line — January showed where it stops extending.¹⁸

METASTATIC

Biomarker-first medicine. What drives the cancer — EGFR, ALK, ROS1, HER2, MET — decides the plan more than any label.^{26,27,28}



WHAT TO HOLD ONTO

- Three stages, three games — one drug almost never serves all three.
- The largest patient group sits in the no-driver lane — and that is where the next fight happens.^{5,15}
- Negative results move practice too: BR.31 settled an adjuvant question for good.¹⁸

Small cell lung cancer — and where the field is heading

SCLC is small in numbers and fast in biology. 2025 gave it new anchors; 2026 will test them.



SCLC — THREE ANCHORS

- Tarlatamab: the DLL3 bispecific reached traditional approval — second line and beyond. ⁸
- Lurbinectedin + atezolizumab: maintenance approval for extensive-stage disease. ^{20,21}
- Risvutatug rezetecan: the B7-H3 ADC picked up orphan designation — the ADC wave is at the door. ¹⁹



WHERE 2026 IS HEADING

- Bispecifics walking forward — from the last line toward the first. ⁵
- Next filings: neladalkib (ALK+) planned for H1; a zidesamtinib expansion follows in H2. ⁶
- HARMONi-3: ivonescimab against pembrolizumab + chemotherapy — the first-line fight. ⁵

“The pattern of the decade: a new mechanism arrives in the last line, proves itself, and walks forward. Ivonescimab is mid-walk. Tarlatamab has arrived. The ADCs are at the door.”

THE ARC TO WATCH — VIGILLINE ANALYSIS

The wave: what the FDA cleared before January

The heaviest approvals landed in late 2025 — which is why January's story is filings, not celebration.

DATE	ASSET	SETTING	STATUS	SOURCE
Oct 2, 2025	Lurbinectedin + atezolizumab	ES-SCLC · maintenance after first-line induction	APPROVED	20,21
Nov 19, 2025	Tarlatamab-dlle (Imdeltra)	ES-SCLC · second line and beyond	TRADITIONAL	8
Nov 19, 2025	Sevabertinib (Hyrnuo)	Non-squamous NSCLC · HER2-mutant, pretreated	ACCELERATED	8
Dec 4, 2025	Risvutatug rezetecan	SCLC · B7-H3 antibody-drug conjugate	DESIGNATION	19
Dec 17, 2025	Amivantamab + hyaluronidase	EGFR-mutant NSCLC · subcutaneous formulation	APPROVED	25,1

How to read this table. A designation is not an approval; an accelerated approval can be confirmed or withdrawn; a traditional approval is the durable kind. January produced none of the three — and that is normal. Approvals arrive in bursts. ⁴

JANUARY ADDED TWO FILES

FILED

IVONESCIMAB — BLA accepted Jan 29 · decision Nov 14⁵

FILED

ZIDESAMTINIB — NDA accepted Jan 12 · decision Sep 18⁶

A filing starts a review — it is not a verdict. The decision dates are commitments, not outcomes.

Late-2025 items, dated. The Dec 4 designation and the Dec 17 approval entered the cycle through January's coverage — every item above carries its own record on slides 19–20. ^{1,19,25}

The lanes in motion: NSCLC by biomarker

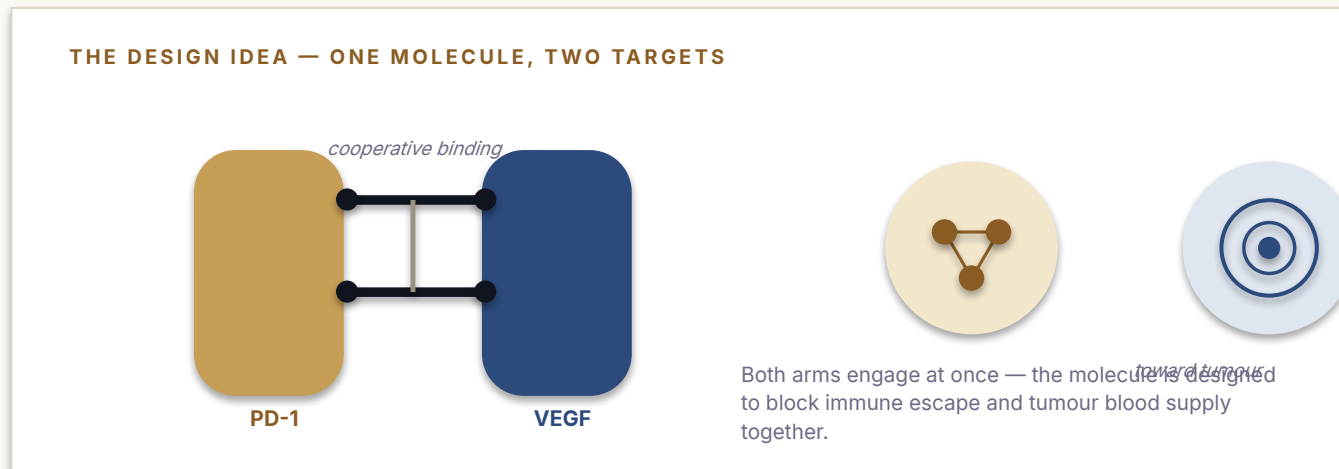
For most metastatic patients, the first question stopped being 'chemo or not'. It is 'what drives this tumour'.

LANE	THE FDA-CLEARED ANCHORS	RECENT MOVEMENT	SOURCE
EGFR	Osimertinib ± chemotherapy first line; amivantamab for exon 20 insertion	Ivonescimab filed for the post-TKI niche (Jan 29); subcutaneous amivantamab available	28,27,5,25
ALK	Alectinib cleared for adjuvant use after resection — targeted therapy in early stage	Neladalkib NDA planned for the first half of 2026	26,6
MET	Tepotinib holds traditional approval for exon 14 skipping	Quiet lane in January — the map holds	30
HER2	Sevabertinib accelerated for HER2-mutant, non-squamous disease	Zongertinib follows on an expedited national-priority track	8,11
ROS1	Next-generation, brain-penetrant agents for TKI-pretreated patients	Zidesamtinib NDA accepted (Jan 12) — decision targeted September 18	6
NO DRIVER	Chemotherapy + checkpoint inhibition — the backbone for the largest group	Ivonescimab aims here next (HARMONi-3) after its EGFR-lane filing	5,15

What the map shows. For most metastatic patients the first question stopped being 'chemo or not' — it is 'what drives this tumour'. The lanes are the strategy. 26,27,28

Ivonescimab reaches the FDA — and a November date lands

Accepted for filing on January 29: the first Western review of a PD-1 × VEGF bispecific in lung cancer.



- **What it is** — a PD-1 × VEGF bispecific antibody, engineered so both targets are engaged by one molecule. ⁵
- **Where it filed** — EGFR-mutated NSCLC after targeted treatment stops working (the HARMONi study). ⁵
- **Why it matters** — the first Western filing for the class, after a deep run of China-led trials. ⁵

THE FILING, PRECISELY

Nov 14

2026 · PDUFA GOAL DATE

Accepted for filing January 29, 2026. ⁵

Decision targeted for November 14, 2026. ⁵

“PDUFA Goal Action Date of November 14, 2026.” ⁵

[SUMMIT THERAPEUTICS](#) — JANUARY 29, 2026

What November 14 decides — and why the room was watching

An accepted filing is the start of a sentencing — three questions the review will settle.

1

Does the evidence travel? The review weighs global and China-first datasets together — how far the data generalise is the FDA's call, not the sponsor's.⁵

2

How does the survival story resolve? Progression-free data convinced reviewers to accept the file; overall survival will decide what follows it.⁵

3

What does a yes do to first line? An approval here sharpens HARMONi-3 — ivonescimab against the pembrolizumab standard, head-to-head, in the biggest market.⁵

THE STAKE — IN ONE SENTENCE

The first Western verdict on PD-1/VEGF — and the opening move in a fight for the largest first-line market in oncology.

ivonescimab
+ chemotherapy

VS

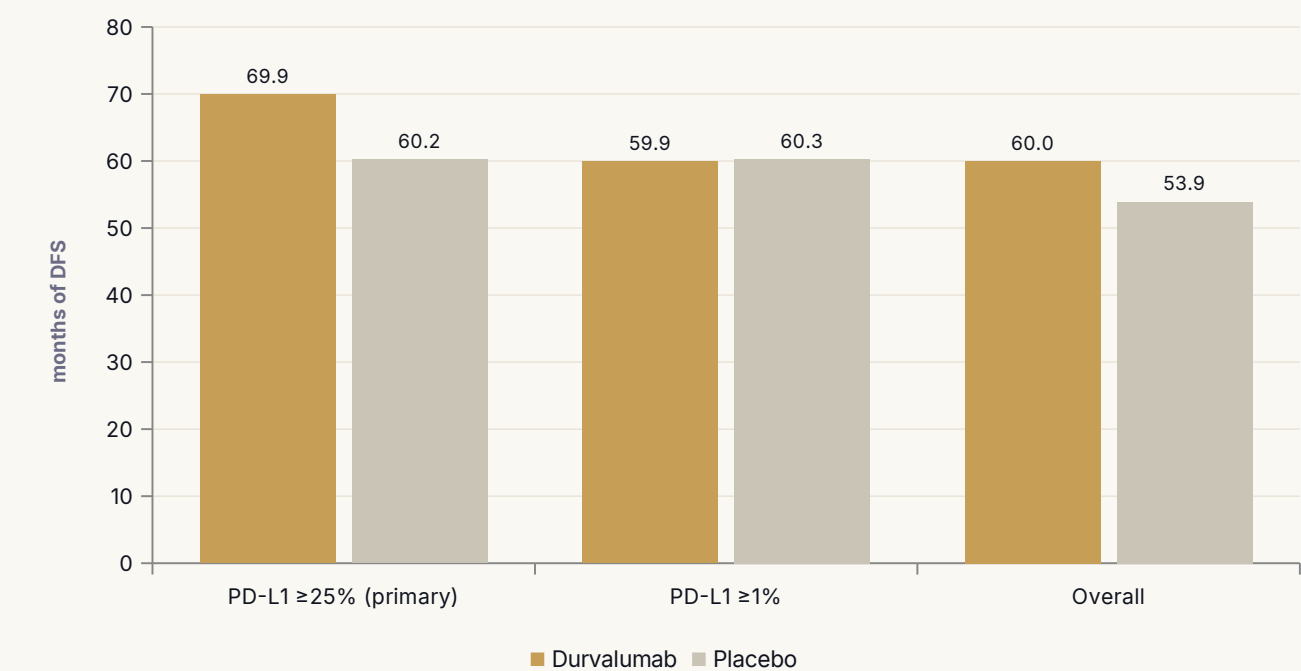
pembrolizumab
+ chemotherapy

HARMONi-3 · first-line metastatic disease — the head-to-head that matters most.⁵

Not a verdict yet. Acceptance starts a review — nothing about care changes today. The room is watching because the date is now real.⁵

BR.31: the adjuvant miss, in numbers

Adjuvant durvalumab after complete resection did not improve disease-free survival. Published January 14.



PRIMARY ENDPOINT MISSED • P = .64

Durvalumab remains standard in unresectable stage III — this result does not change that. It settles the post-surgery question for this design.

PD-L1 ≥25% (primary)	HR 0.93
PD-L1 ≥1%	HR 0.99
Overall	HR 0.89

What happened. Median disease-free survival: 69.9 months with durvalumab versus 60.2 with placebo — not significant (HR 0.93; P = .64). [Full result ↗](#)

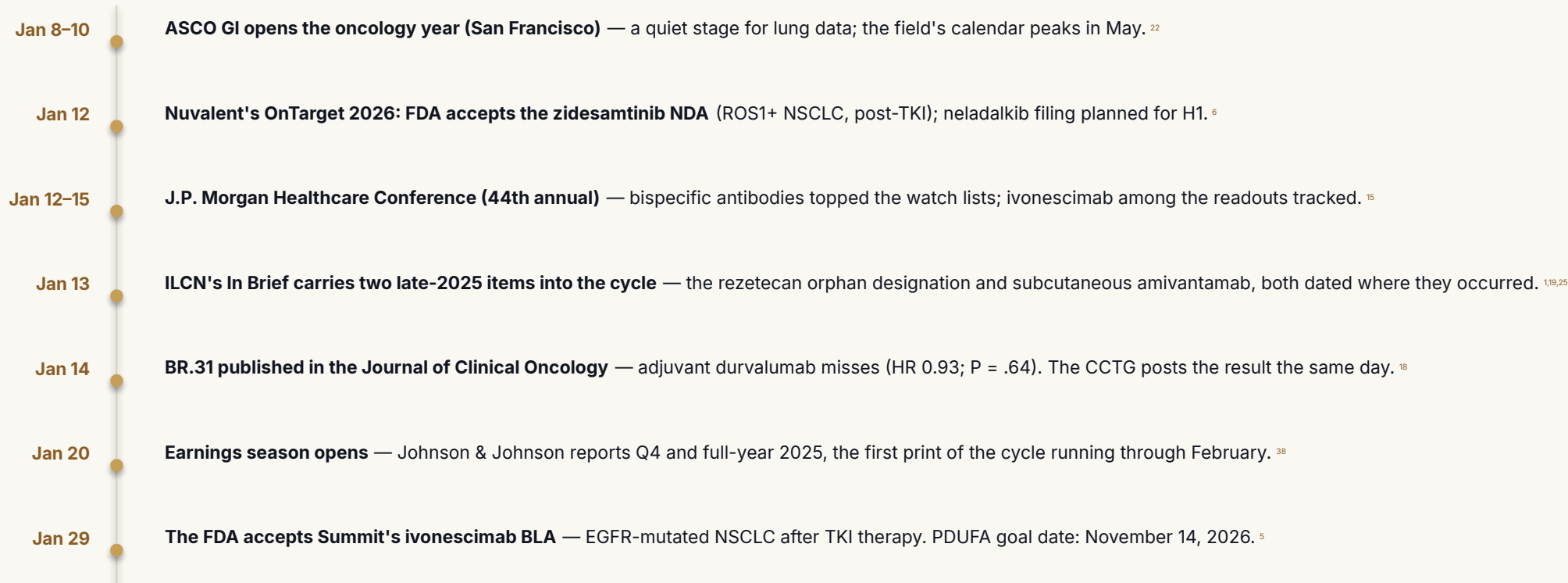
Why it matters. One clean negative: the adjuvant question is answered — practice moves on.¹⁸

“These agents do not directly kill cancer cells... the task moving forward is to understand the biology and ask how best to integrate these agents.”¹³

PROF. GLENWOOD GOSS — BR.31 INVESTIGATOR

January 2026: the timeline

Seven dated entries — public records only, in the order they happened.



How we date events. Approvals are dated to the regulator's action, publications to the journal — and late-2025 items the trade press carried into January are labelled as such. The timeline never mixes them. ¹

The money week: what JPM set up for the year

January 12–15, San Francisco — the capital-markets meeting that frames the field's agenda for 2026.



WHAT THE ROOM WAS WATCHING

- **Bispecifics as the new centre of gravity** — the PD-1/VEGF class, ivonescimab above all, dominated the watch lists. ¹⁵
- **The “next Keytruda” question, asked out loud** — who builds the successor franchise to the biggest oncology drug of the decade? ¹⁵
- **Small-cap precision plays** — Nuvalent put its 2026 plan on the record: zidesamtinib under review, neladalkib to be filed, runway into 2029. ⁶
- **Attention migrates to filings** — capital now chases pending decisions as much as launched products.



THE REPORTING CYCLE OPENS

Johnson & Johnson — Q4 / FY2025, January 20. The first print of the cycle that carries the field's commercial story through February. ³⁸

NEXT IN THE CYCLE

AstraZeneca, Merck and peers report into early February — the launch-by-launch commercial picture lands in Issue 02.

What to watch next. The filing queue: neladalkib (ALK+) planned for H1; a zidesamtinib expansion in H2 — and February's earnings prints on top. ⁶

Three signals from the research front

Better selection, broader mechanisms, faster reviews — three ways the same thing happens.

1



A blood test to read tarlatamab

A small January study suggests a marker on circulating tumor cells may predict which SCLC patients get a lasting response to tarlatamab — a step toward matching the bispecific to the patients it serves best. ³²

"Isolating cancer cells from the blood has tremendous potential to guide immune-related cancer therapies." — investigator comment, via The ASCO Post

2



The B7-H3 lane gathers speed

Risvutatug rezetecan's SCLC orphan designation headlined a widening antibody–drug–conjugate push into small cell disease — a class that spent a decade waiting for one. ^{19,1}

3



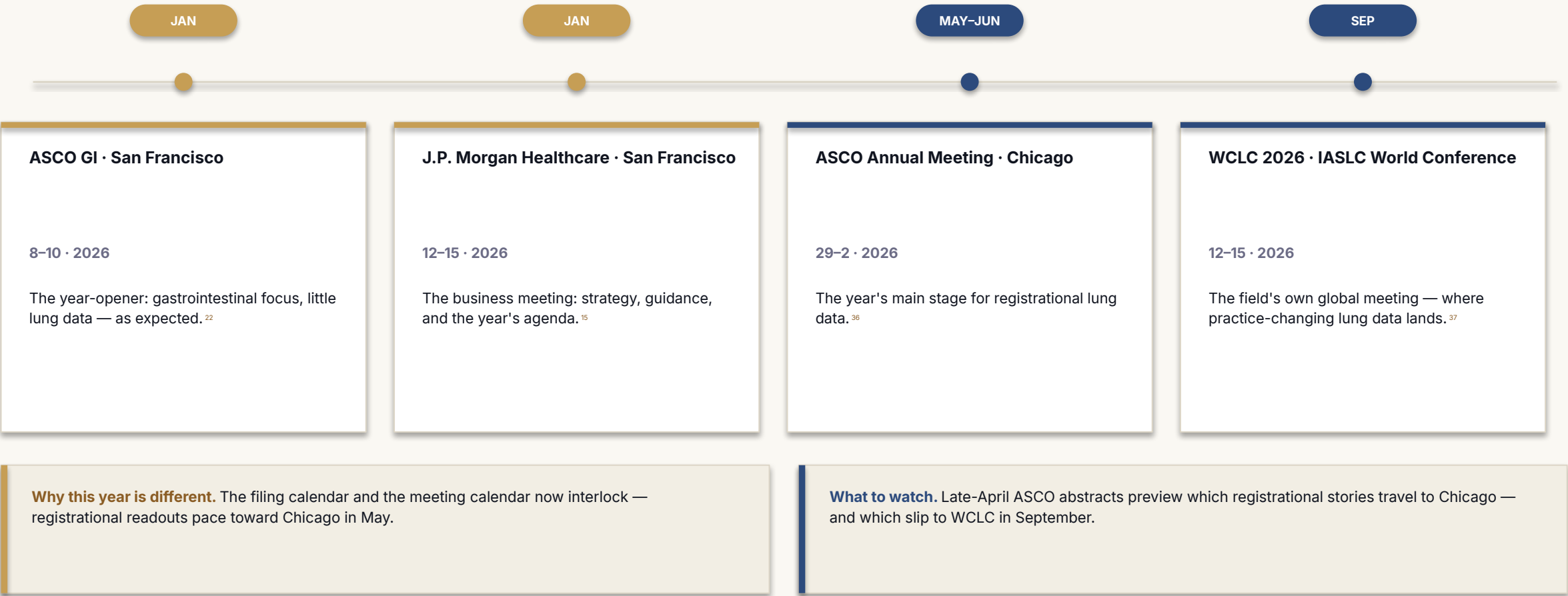
Speed itself becomes a strategy

The FDA's national-priority voucher pilot is rewriting review economics for select drugs — a 44-day turnaround was cited on the pilot's second approval. ¹¹

Why the trio belongs together. Better patient selection (CTCs), broader mechanisms (ADCs), and faster reviews (priority vouchers) are three versions of the same event: decisions arriving sooner, for narrower and better-defined populations. ^{32,11}

The calendar that shapes the news

Four anchor dates set the rhythm of lung-cancer news — everything else rides between them.



Two dates that matter

Acceptances became dates in January — these are the decisions the field will trade on all year.

SEP 18 2026

Zidesamtinib (ROS1+)

TKI-pretreated ROS1-positive NSCLC — adds a next-generation, brain-penetrant option in a lane defined by first-generation resistance.⁶

NOV 14 2026

Ivonescimab + chemotherapy

EGFR-mutated NSCLC, post-TKI — the first US decision on a PD-1/VEGF bispecific, and a preview of the first-line fight.⁵

Watch the milestones. H1 — neladalkib (ALK+) NDA planned.⁶ · H2 — a zidesamtinib expansion planned.⁶ · Through the year — HARMONI-3 pacing toward the first-line readout, the head-to-head that matters most.⁵

How to engage. Dates in this business are commitments, not outcomes — track them as checkpoints, not verdicts: what files, what reads out, and what the reviews decide, in that order.

Screening: who should be asking the question

Most of this issue is about treatment. The highest-leverage moment happens before treatment — finding lung cancer early.



Age 50 to 80.



A smoking history of 20 pack-years or more — a pack a day for 20 years, or two packs a day for 10.



Currently smoking, or quit within the past 15 years.

The bottom line. If you meet the criteria, ask your clinician whether low-dose CT screening is right for you. If not — no action needed. The recommendation is deliberately narrow. ²⁴

WHY IT'S WORTH THE CONVERSATION

Lung cancer kills more Americans than any other cancer — an estimated **124,730 deaths in 2025** — largely because most cases are found after symptoms begin. ^{24,11}

Screening finds disease earlier, when treatment has the best odds. It is a conversation to start with a primary-care clinician.



PACK-YEAR One pack a day for a year. 20 pack-years means a pack a day for 20 years — or two packs a day for 10.

LOW-DOSE CT A fast chest scan at a low radiation dose; no contrast dye, no needles.

SCREENING ≠ DIAGNOSTIC Screening is for people without symptoms, to find problems early.

January's news, translated

The month's three stories, in plain words — for patients and families.



THE QUIET

No new medicine was approved this month. That is normal — approvals come in bursts, not on a schedule. Nothing already available was taken away, and nothing about existing care changed.⁴



THE FILING

A promising new option started its review — decision expected by mid-November. Ivonescimab is being reviewed for people with EGFR-mutated lung cancer whose current targeted treatment has stopped working.⁵



THE STUDY

One drug did not help in one specific situation: adding durvalumab after surgery did not keep cancer from returning sooner in the group studied. The same drug remains standard where it is already used.¹⁸

The one-line version. Quiet month on approvals, a new option in review, one open question settled — that is what January changed, for everyone.

The size of the problem — and of the room

Two numbers frame the disease; one frames the best chance to catch it early.



THE TWO NUMBERS, TO SCALE



Why 'upstream' matters. Approvals change labels; filings change expectations; a negative trial changes practice. January delivered one of each — which is why the month was quieter than it reads. ^{5,6,18}

Every claim, checkable — sources 1–19

Every entry opens the original public record. Primary sources take priority; trade coverage only where primary records do not exist.

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Sources 20–38 — and where to find us

Click any title or link to open the source. Superscripts throughout this issue resolve to these entries.

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